African and Indigenous American ancestry shape plasma CNS biomarker baselines within and across populations, modifying Alzheimer's disease trajectories

Eden R. Martin[1], Tianjie Gu[1], Olabode Omotoso[2], Lissette Gomez[1], Maryanella Illanes-Manrique[3], Kazeem Sanjo Akinwande[4], Patrice Whitehead[1], Michael Cuccaro[1], Alfred Kongnyu Njamnshi[5], Njideka Okubadejo[6], Jean Ikanga[7], Damas Andrea[8], Michelle Nichols[9], Olusegun Baiyewu[4], Mayowa Owolabi[10], Rajesh Kalaria[11], Brian Kunkle[1], Jeffery Vance[1], Christiane Reitz[12], Giuseppe Tosto[12], William Bush[13], Jonathan Haines[13], Adesola Ogunniyi[4], Goldie Byrd[14], Farid Rajabli[1], Rufus Akinyemi[4], Mario Cornejo-Olivas[3], Margaret A. Pericak-Vance[1], Anthony Griswold[1]

[1] Hussman Institute for Human Genomics, University of Miami, Miami, FL, USA; [2] Neurology, University of Ibadan, Nigeria; [3] Instituto Nacional de Ciencias Neurológicas, Lima, Peru; [4] Institute for Advanced Medical Research and Training, University of Ibadan, Nigeria; [5] Brain Research Africa Initiative, Yaoundé, Cameroon; [6] Medicine, University of Lagos, Nigeria; [7] Psychiatry, University of Kinshasa, DRC; [8] Psychiatry, Mirembe National Mental Hospital, Dodoma, Tanzania; [9] Health Sciences and Rehabilitation, Medical University of South Carolina, Charleston, SC, USA; [10] Translational and Clinical Research Institute, University of Ibadan, Nigeria; [11] Translational and Clinical Research Institute, Newcastle University, UK; [12] Vagelos College of Physicians and Surgeons, Columbia University, New York, NY, USA; [13] Population and Quantitative Health Sciences, Case Western Reserve University, Cleveland, OH, USA; [14] Biostatistics and Data Science, Wake Forest School of Medicine, Winston-Salem, NC, USA

Board 1003W · emartin1@miami.edu

THE QUESTION

A biomarker reads differently in two populations: disease, ancestry, or recruitment site? Does it change the test?

THE ANSWER

Ancestry moves the baseline

- 62 of 126 proteins differ; African vs HI-WH 49, all the same sign in both lots
- a known variant accounts for CHIT1 (78%) and part of IL6R (45%)

Ancestry rarely moves the AD signal

- p-tau rises with AD in every group, in both lots
- one BD-pTau-217 cut: same share of normals above it, fewer clinical AD at most African sites

Few regional differences survive site

- Peruvian and Ecuadorian vs Caribbean 10; East vs West Africa 5
- 6 of the 15 immune

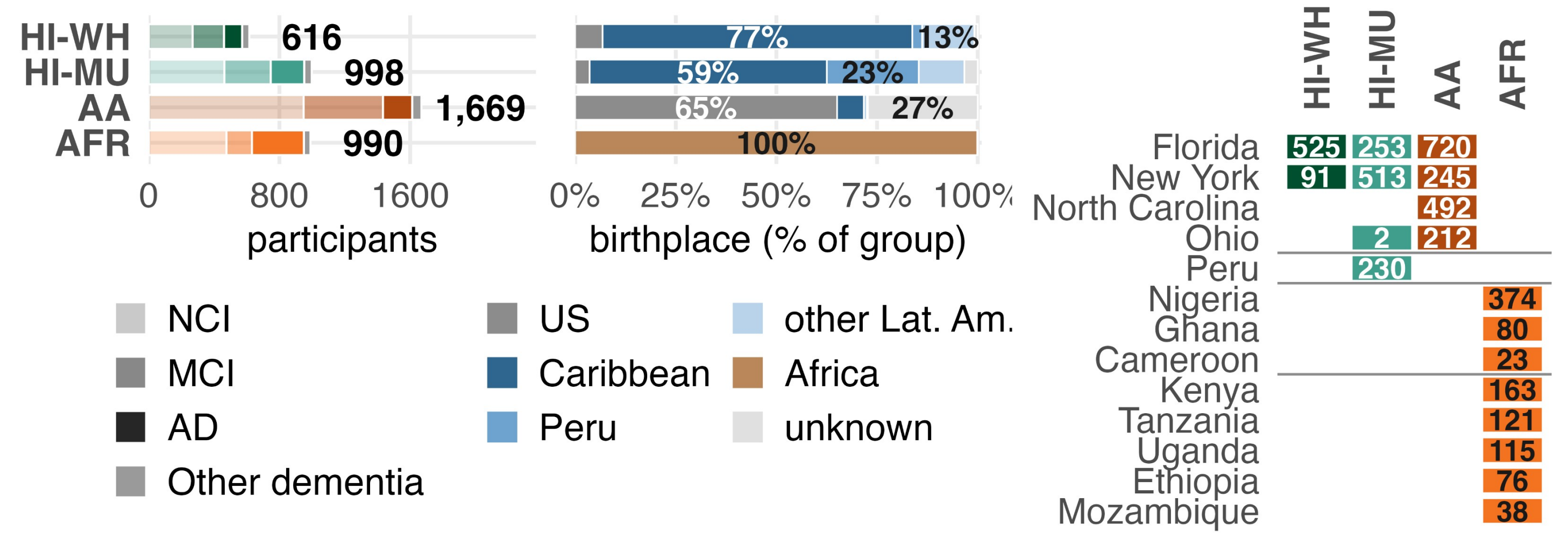
WHO AND HOW

4,273 participants

22 sites

50 countries of birth

126 plasma proteins



	HI-WH	HI-MU	AA	AFR
Florida	525	253	720	
New York	91	513	245	
North Carolina			492	
Ohio		2	212	
Peru		230		
Nigeria				374
Ghana				80
Cameroon				23
Kenya				163
Tanzania				121
Uganda				115
Ethiopia				76
Mozambique				38

female 72 / 74 / 79 / 56%; median age 73 / 71 / 71 / 72 (HI-WH / HI-MU / AA / AFR)

■ **HI-WH** Hispanic, European descent · 3 sites
■ **HI-MU** Hispanic, mixed origin · 6 sites
■ **AA** African American · 6 sites
■ **AFR** African · 13 sites
22 sites in all: five serve more than one group

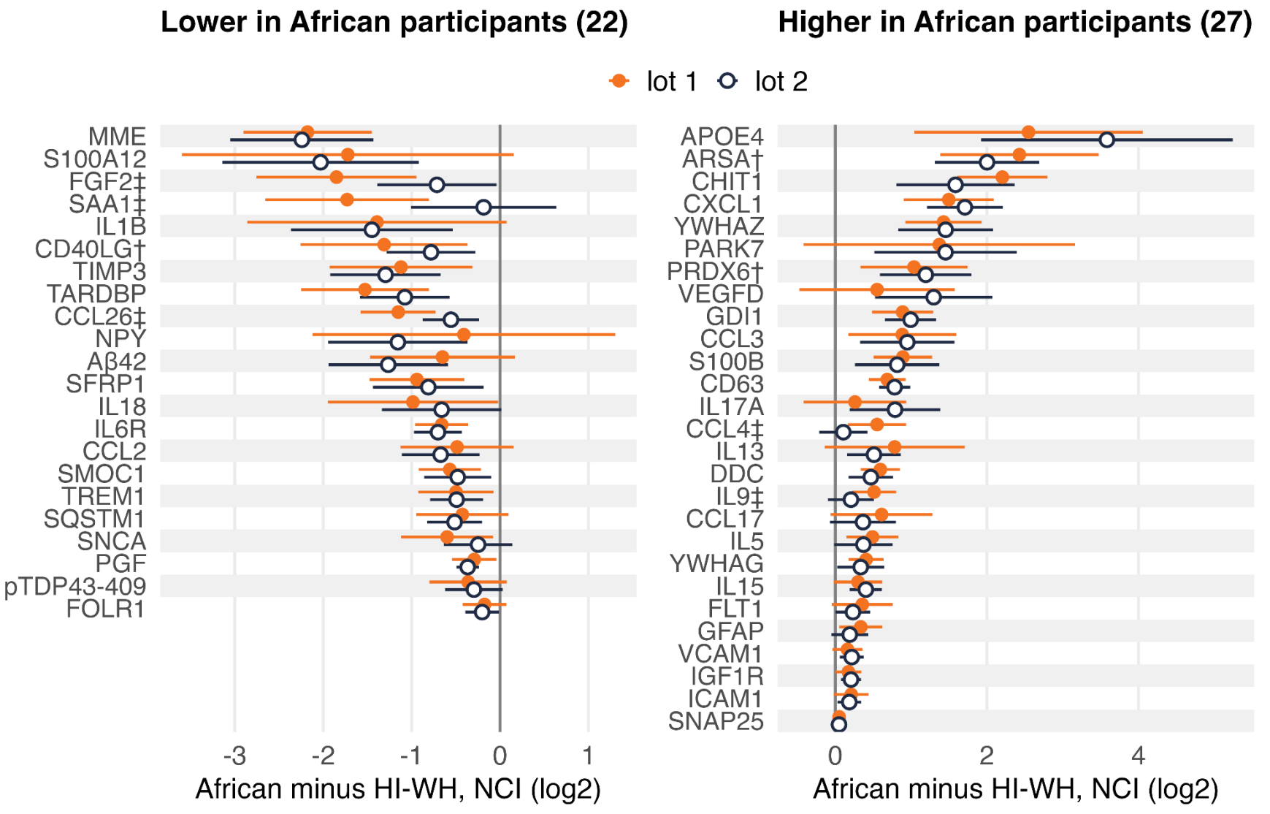
- Alamar NULISAseq CNS panel: 131 targets, 126 proteins after the hemolysis filter
- 2 reagent lots, each fitted alone: lot 1, 2,467 participants (2025); lot 2, 1,806 (2026)
- model: ancestry, sex, age, diagnosis, reads and a blood-cell score, with site and assay run as random effects; ancestry p-values by parametric bootstrap; full methods in the supplement

APOE ε4: THE COUNTER-EXAMPLE

- among the cognitively normal, no evidence that the ε4 effect on p-tau differs by group (pTau-217 p 0.75 in lot 1, 0.55 in lot 2)
- the group difference in the ε4 slope is a lot-1 result that comes with the diagnosis mix (clinical AD: African 38%, AA 9% of those fits)
- ε4 dose at NCI 0.22 / 0.20 / 0.44 / 0.49; figure in the supplement

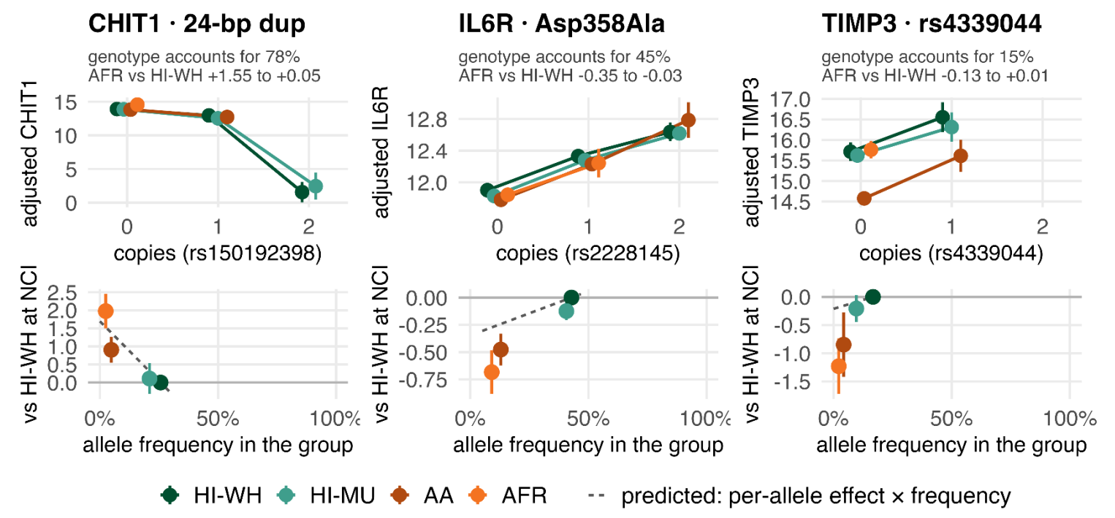
1. ANCESTRY MOVES THE BASELINE

All 49 African vs HI-WH differences point the same way in both lots



- 62 of 126 differ; vs HI-WH at NCI: African 49, AA 38, HI-MU 3
- ρ 0.93 between lots, size ratio 0.99; 19 above 2-fold; lots disagree for 5 (‡), reagent changed for 3 (†): lot 1 shown
- HI-MU, Peruvian and Ecuadorian vs Caribbean: 10, all diagnoses, inside one site (section 3)

The CHIT1 difference is the allele frequency; for IL6R, about half of it

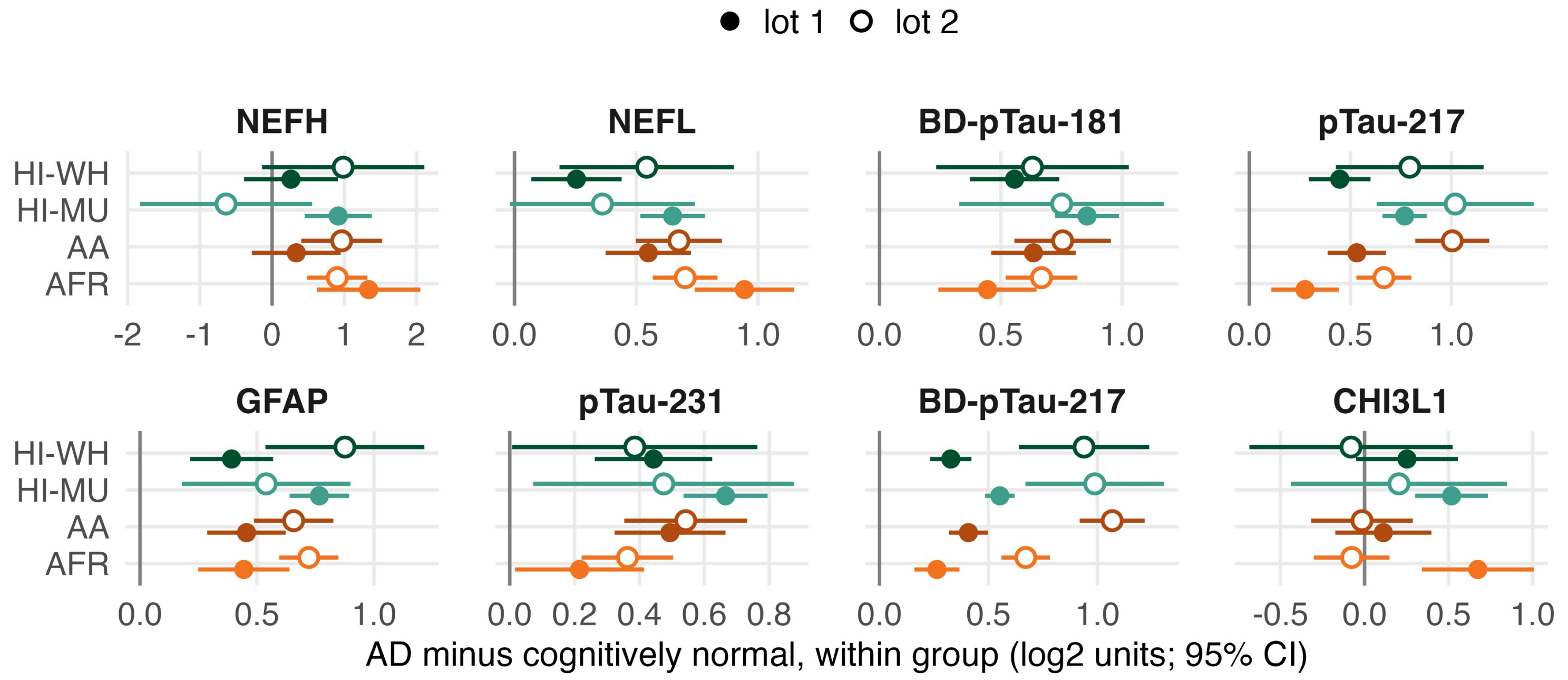


Top: cognitively normal participants with WGS, by allele dose within group; values in log2.
Bottom: each group's difference from HI-WH (main model); dashed = per-allele effect × allele frequency.
% = share of the African vs HI-WH difference the protein's confirmed variants predict (IL6R: with Duffy-null).
Subtitle: the African vs HI-WH difference among these participants, before and after the genotype enters the model; both lots in one fit, with a lot term. Dashed line = the variant shown alone.
All African participants with WGS are in lot 2: their top-panel level rests on the lot adjustment.

Top: by allele dose within group, cognitively normal with WGS. Bottom: each group vs HI-WH against its frequency; dashed = per-allele effect × frequency; % = whole-cohort share. Before / after the genotype enters the model, sequenced participants, both lots in one fit with a lot term: CHIT1 +1.55 to +0.05 log2, IL6R −0.35 to −0.03, TIMP3 −0.13 to +0.01 (cohort −1.23). WGS 2,278 of 4,273; 30 of the 49 have no variant in the WGS set, 9 have one that does not move the protein.

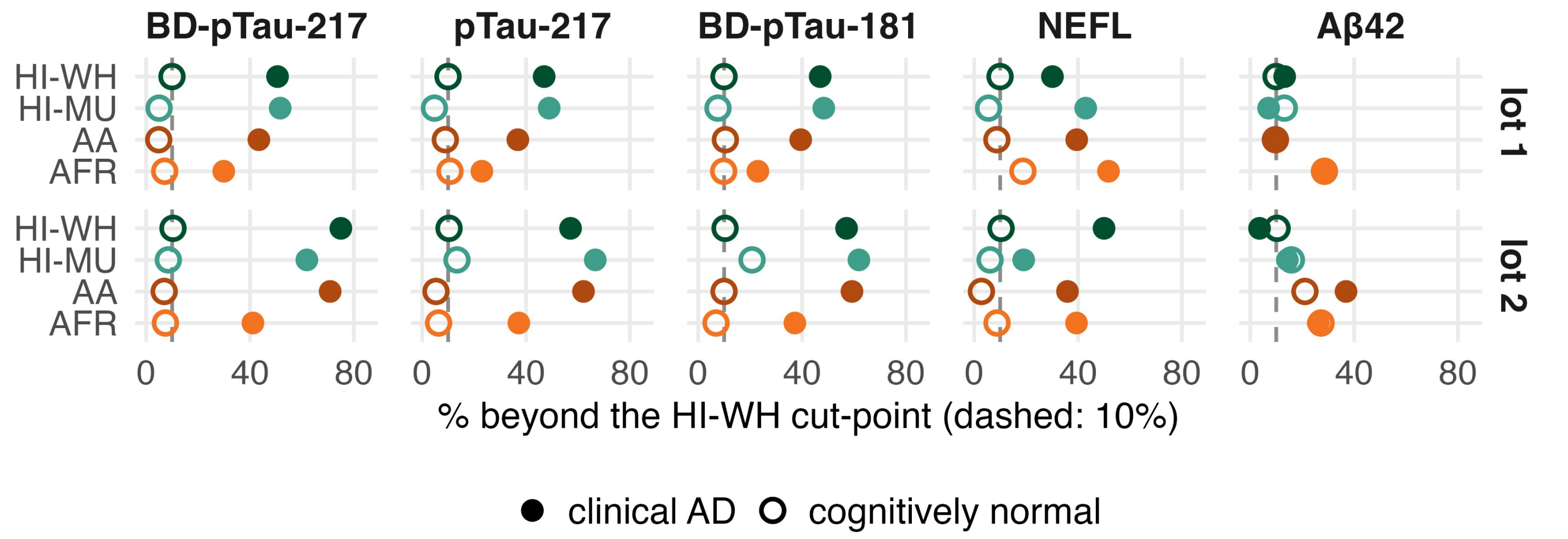
2. ANCESTRY RARELY MOVES THE AD SIGNAL

p-tau rises with AD in every group, in both lots; BD-pTau-217 rises least in African participants



- 12 group × diagnosis interactions of 126; BD-pTau-217 holds in lot 2 alone (p 0.002), pTau-217 and NEFL do not

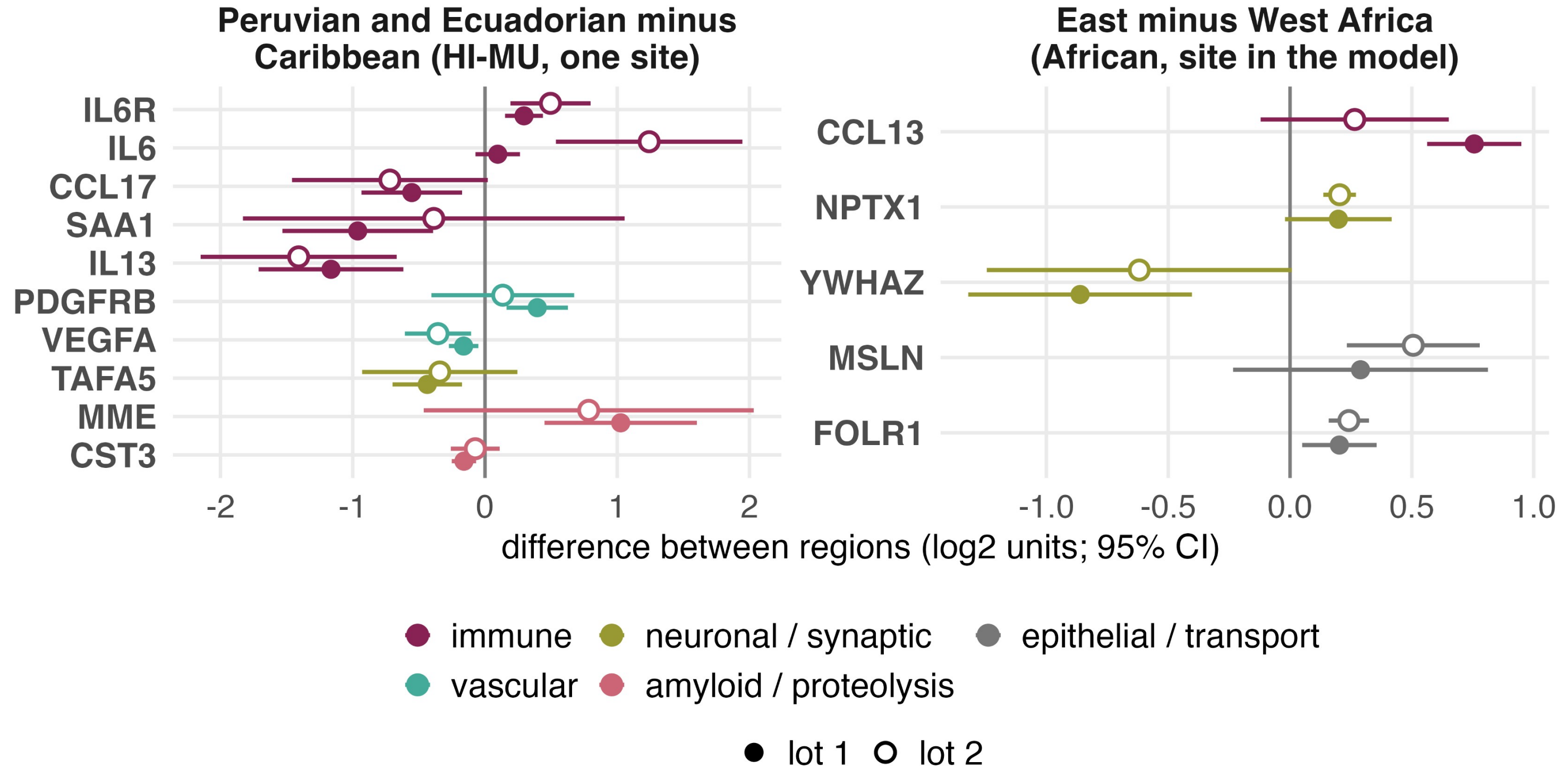
One BD-pTau-217 cut flags 5 to 10% of normals in every group, and fewer clinical AD at most African sites



Cut: 90th percentile of HI-WH cognitively normal, within lot (10th for Aβ42). BD-pTau-217, clinical AD above the cut: African 30% vs 43 to 52% (lot 1), 41% vs 62 to 75% (lot 2; HI-WH 28 AD / 58 NCI, HI-MU 21 / 82); by site in lot 2, 5 of 8 African sites 31 to 44%, 3 (50 to 75%) with the other groups' 50 to 76%; the same pattern on pTau-217 and BD-pTau-181.

3. REGIONS WITHIN A GROUP

With site in the model, 10 proteins differ Peruvian and Ecuadorian vs Caribbean and 5 East vs West Africa; 6 of the 15 are immune



- 197 of 260 are at the Peru site: compared within one New York site
- IL6R higher in Peruvian and Ecuadorian, as rs2228145-C predicts (gnomAD: Latino 0.53, European 0.40, African 0.13)

CAVEATS

- clinical diagnosis, not amyloid-confirmed; one draw per person
- groups: self-identified (US) or born in Africa; no genetic PCs
- African: own 13 sites, one assay run in lot 1
- handling, storage, diagnostic workup differ by site

SUPPLEMENT

detailed methods and the model, the model-term table, the ε4 figure, the six-arm interaction matrix, the five-panel cohort figure, per-lot tables, the full genotype table · github.com/emartin-um/ASHG2026-supplement

WHAT THIS MEANS

Normals cross the cut-point equally in every group; clinical AD does not.

p-tau: confirm AD with amyloid PET or CSF in every group before recalibrating the cut-point · plasma Aβ42 and Aβ42/40 do not separate AD in any group · a one-site population difference is a site difference until shown otherwise

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